

THE THREE FOUNDATIONAL CONSTITUTIONS OF SUN-NAH REMEDIES ACADEMY

The Complete Master Document

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“The best of you are those who learn the Qur’an and teach it.” — Sahih al-Bukhari

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PART I

THE SUNNAH REMEDIES ACADEMY DESIGN BIBLE

The Visual & Brand Constitution

PREAMBLE

This Design Bible constitutes the supreme visual authority of Sunnah Remedies Academy. Every pixel, pigment, proportion, and printed piece that represents the Academy must derive its legitimacy from this document. No course material, certificate, social media post, clinical form, or instructor presentation may depart from the standards herein without written approval from the Academy Design Council.

This Bible exists to ensure that whether a student encounters Sunnah Remedies Academy in a Hijama clinic in Kuala Lumpur, a Prophetic Medicine workshop in London, or an online herbal certification in Toronto, they experience one unmistakable, coherent, and spiritually grounded institutional identity.

CHAPTER 1: THE ACADEMY SEAL & IDENTITY ARCHITECTURE

1.1 The Primary Academy Seal

The Academy Seal is the sovereign mark of institutional authority. It appears on:

- Official certificates and diplomas
- Legal and regulatory correspondence
- Academic transcripts and credential verifications
- High-level partnership agreements

Specifications:

- **Form:** A geometric medallion combining an eight-pointed star (representing the eight categories of Prophetic Medicine) enclosing a stylized calligraphic rendering of the Academy name in bespoke Arabic-Latin hybrid typography.
- **Color:** Rendered exclusively in Academy Gold (Pantone 871 C / CMYK 0, 17, 50, 30) on Academy Navy (Pantone 289 C / CMYK 100, 56, 0, 56) or on white with gold foil embossing.
- **Minimum Size:** 15mm in print; 48px in digital.
- **Clear Space:** No other element may encroach within a margin equal to the height of the central star point.

1.2 The Wordmark

The primary wordmark reads “SUNNAH REMEDIES ACADEMY” in the bespoke typeface **Sunnah Serif**, with the tagline “Healing by the Book and the Sunnah” set in **Sunnah Sans** beneath, separated by a 0.5pt hairline rule.

1.3 The Course Emblem System

Each discipline carries a subsidiary emblem that must always appear in relationship to the primary seal, never as a standalone logo:

Discipline	Emblem Motif	Color Accent
Hijama	Cupping vessel with crescent steam	Copper
Prophetic Medicine (Tibb Nabawi)	Olive branch with date palm	Olive Green
Herbal Medicine	Mortar and pestle with leaf motif	Sage
Nutrition	Bowl with grain and honey dipper	Amber
Instructor Training	Open book with lantern	Deep Indigo

Rule: Course emblems may never exceed 60% of the primary seal’s dimensions when paired. They must always appear below or to the right of the primary seal, never above or overlapping.

CHAPTER 2: THE SACRED GEOMETRY & SPATIAL SYSTEM

2.1 The Proportional Grid

All Academy layouts derive from a **12-column modular grid** based on the golden ratio (1:1.618). The grid is not merely aesthetic; it reflects the harmony (tawazun) central to Islamic design tradition.

- **Base Unit:** 8.4mm (print) / 24px (digital)
- **Gutter:** 1 base unit
- **Margin:** 2 base units minimum

2.2 The Breath Margin

Every page, slide, and screen must contain a “breath margin”—an area of intentional negative space equal to at least 15% of the total canvas. This space represents the spiritual pause (sakina) that distinguishes Academy materials from cluttered commercial design.

2.3 The Mihrab Frame

For formal documents, certificates, and presentation title slides, a subtle **mihrab arch frame** may be used as a top border element. This is not decorative; it orients the reader toward the sacred purpose of the content.

Specifications:

- Arch height: 8% of page height
 - Line weight: 0.25pt
 - Color: Academy Gold at 30% opacity
 - No fill; purely linear
-

CHAPTER 3: THE TYPOGRAPHIC HIERARCHY

3.1 The Academy Typeface Family

Primary Serif: Sunnah Serif

- Usage: Headlines, chapter titles, certificate names, formal addresses
- Weights: Light, Regular, Medium, Bold
- Character: High contrast between thick and thin strokes, echoing classical Ottoman naskh proportions
- Numerals: Old-style proportional, always tabular in data tables

Primary Sans: Sunnah Sans

- Usage: Body text, UI elements, captions, clinical forms
- Weights: Light, Regular, Medium, SemiBold, Bold
- Character: Humanist sans with slightly condensed proportions for efficient information density
- Numerals: Lining proportional

Secondary: Sunnah Arabic

- Usage: Arabic text, Quranic citations, hadith references, transliteration
- Weights: Regular, Bold
- Character: Modernized traditional naskh with optimized screen rendering

Accent: Sunnah Script

- Usage: Signatures, personal dedications, decorative calligraphic elements
- Character: Refined thuluth-inspired script, never used for body text or functional labels

3.2 Type Scale

All typography follows a **modular scale of 1.25** (Major Third):

Level	Size (Print)	Size (Digital)	Weight	Usage
H1	42pt	48px	Sunnah Serif Bold	Page titles, certificate names
H2	33.6pt	38.4px	Sunnah Serif Medium	Section headers
H3	26.8pt	30.7px	Sunnah Sans SemiBold	Subsection headers
H4	21.5pt	24.6px	Sunnah Sans Medium	Module titles
Body	12pt	16px	Sunnah Sans Regular	Primary reading text
Caption	9.6pt	12.8px	Sunnah Sans Light	Annotations, references
Micro	7.7pt	10.2px	Sunnah Sans Light	Legal text, disclaimers

Rule: Body text must never be set smaller than 12pt in print or 16px in digital for accessibility compliance.

CHAPTER 4: THE COLOR COVENANT

4.1 The Primary Palette

These colors are sacred to the Academy identity. They may not be approximated, substituted, or altered.

Color Name	Pantone	CMYK	HEX	RGB	Usage
Academy Navy	289 C	100, 56, 0, 56	#041E42	4, 30, 66	Primary background, authority, depth
Academy Gold	871 C	0, 17, 50, 30	#B3A369	179, 163, 105	Excellence, achievement, sacred quality
Academy White	—	0, 0, 0, 0	#F8F6F0	248, 246, 240	Purity, clarity, space
Academy Ink	—	0, 0, 0, 90	#1A1A1A	26, 26, 26	Primary text, gravitas

4.2 The Secondary Palette

These colors support the primary palette without competing for dominance.

Color Name	Pantone	HEX	Usage
Sage	7496 C	#8A9A5B	Herbal medicine, nature, growth
Terracotta	7606 C	#E2725B	Warmth, human touch, community
Deep Indigo	274 C	#1E2A4A	Instructor level, wisdom, depth
Sand	7499 C	#D4C4A8	Neutrality, parchment, tradition
Olive	5743 C	#4A5D23	Prophetic medicine, barakah

4.3 Color Usage Doctrine

- **60-30-10 Rule:** 60% primary (Navy/White), 30% secondary, 10% accent (Gold)
- **No Pure Black:** Academy Ink (#1A1A1A) replaces pure black in all contexts to reduce visual harshness
- **Gold Discipline:** Academy Gold may only appear on Navy, White, or Deep Indigo backgrounds. Never on Sage, Terracotta, or Olive.
- **Contrast Minimum:** All text must meet WCAG 2.1 AA contrast ratios (4.5:1 for body, 3:1 for large text)

CHAPTER 5: THE PHOTOGRAPHIC & ILLUSTRATIVE LANGUAGE

5.1 The Photography Covenant

All Academy photography must embody what we call “**Sacred Clarity**”—images that are:

- **Clean:** Uncluttered backgrounds, shallow depth of field, clinical precision
- **Warm:** Natural lighting, human warmth, skin tones rendered accurately
- **Dignified:** No sensationalism, no gratuitous clinical imagery, no commercial flashiness
- **Contextual:** Hijama cups shown with hands performing the sunnah, herbs shown with reference to classical texts, not isolated on white like product shots

Prohibited:

- Stock photography with visible watermarks
- Images of patients without explicit consent documentation
- Graphic clinical imagery (blood, open wounds) in promotional materials
- Artificial filters or heavy color grading that departs from the Academy palette

5.2 The Medical Illustration Standard

All anatomical and procedural illustrations follow the “**Classical Precision**” style:

- Cross-sectional views rendered in fine line weight (0.5pt–1pt)

- Color coding: Arteries in Academy Gold, veins in Deep Indigo, nerves in Terracotta, lymph in Sage
- Labels in Sunnah Sans Medium, 8pt, with leader lines in 0.25pt Academy Ink
- Background: Academy White with subtle grid lines at 5% opacity
- No 3D rendered “plastic” anatomy; all illustrations must have the quality of classical medical engraving

5.3 The Iconographic Vocabulary

The Academy maintains a proprietary icon library of 240+ icons organized into eight families:

1. **Ritual & Sunnah** (prayer, fasting, cupping, hijama)
2. **Anatomy** (body systems, points, meridians)
3. **Botanical** (herbs, preparation, cultivation)
4. **Clinical** (sterilization, safety, documentation)
5. **Academic** (certification, assessment, graduation)
6. **Digital** (login, download, progress, completion)
7. **Navigation** (menu, search, breadcrumb, pagination)
8. **Status** (success, warning, error, information)

Icon Specifications:

- Stroke weight: 1.5pt
- Corner radius: 0pt (sharp) for clinical/academic; 2pt for community/ritual
- Color: Academy Navy or Academy Ink
- Size: 24px (standard), 48px (feature), 16px (micro)

CHAPTER 6: THE DOCUMENT TEMPLATE SYSTEM

6.1 The Master Document Families

Family A: Academic Publications

- Course textbooks, instructor manuals, student workbooks
- Format: A4 or US Letter, perfect-bound or spiral
- Template: 12-column grid, 2.5cm margins, running headers in Sunnah Serif Small Caps
- Page numbers: Bottom center, Sunnah Sans 9pt, preceded by chapter number

Family B: Clinical Documentation

- Patient intake forms, consent forms, treatment records, case study templates
- Format: A4, single-sided or duplex
- Template: Clean form grid, ample white space for handwriting, Sunnah Sans 10pt labels
- Color: Academy Ink on Academy White; color used only for section dividers and critical warnings

Family C: Certification & Credentials

- Diplomas, certificates of completion, transcripts, instructor licenses
- Format: A4 or custom 11×14” for wall display
- Template: Mihrab frame top, Academy Seal centered, Sunnah Serif H1 for recipient name
- Paper: 300gsm cotton stock with watermark; gold foil seal application

Family D: Presentation & Teaching

- Slide decks, projection materials, video title cards
- Format: 16:9 widescreen, 1920×1080 minimum
- Template: Breath margin 15%, title in Sunnah Serif H1, content in Sunnah Sans Body
- Background: Academy White or Academy Navy; never gradients or photographs behind text

Family E: Digital Interface

- Learning Management System (LMS), student portal, instructor dashboard
- Format: Responsive web, 320px–2560px width
- Template: 8px base grid, card-based layout, Sunnah Sans for all UI text
- Accent: Gold for primary actions, Sage for success states, Terracotta for warnings

Family F: Promotional & Community

- Social media, event posters, email newsletters, community announcements
- Format: Variable per platform
- Template: Must include Academy Seal or wordmark; color palette strictly observed
- Photography: Sacred Clarity standard; no text overlaid on faces or clinical procedures

6.2 The Typography of Authority

Every document family must include the following “authority block” in the footer or final page:

SUNNAH REMEDIES ACADEMY *Healing by the Book and the Sunnah* [Document ID] |
[Version] | [Date] www.sunnahremedies.academy | [Regulatory number if applicable]

CHAPTER 7: THE DIGITAL DESIGN SYSTEM

7.1 The LMS Visual Architecture

The Academy Learning Management System follows a “**Madrassa Minimalism**” philosophy:

- Navigation: Persistent left sidebar, 240px width, Academy Navy background, white text
- Content area: Academy White background, maximum 720px reading width for optimal line length
- Cards: 1px Academy Ink border at 10% opacity, 8px corner radius, no shadows
- Progress indicators: Thin horizontal bars, Academy Gold fill, no animations that delay content access

7.2 The Video Production Standard

All Academy video content:

- **Opening:** 3-second Academy Seal animation (subtle gold light reveal), no sound
 - **Lower Thirds:** Sunnah Sans Medium, 36pt, Academy White on Academy Navy bar, 80% opacity
 - **Backgrounds:** Academy White or subtle Sand texture; no virtual backgrounds in instructor presentations
 - **Captions:** Sunnah Sans Regular, white with 2px Academy Navy text shadow, bottom center
 - **Closing:** Authority block with QR code to course page, 5 seconds
-

CHAPTER 8: THE MATERIAL & PRINT STANDARD

8.1 The Paper Covenant

- **Academic Publications:** 100gsm uncoated, FSC-certified, cream white (matches Academy White)
- **Clinical Forms:** 90gsm offset, white, laser-printer compatible
- **Certificates:** 300gsm cotton rag, deckle edge optional, watermark of Academy Seal
- **Business Materials:** 350gsm duplex board, matte laminate, gold foil or blind emboss

8.2 The Binding Hierarchy

- **Student Workbooks:** Wire-o binding, allows 180° flat lay for clinical reference
 - **Instructor Manuals:** Hardcover, Smyth-sewn, ribbon marker in Academy Gold
 - **Clinical Protocols:** Ring-bound or laminated single sheets for sterilization compatibility
 - **Certificates:** Unbound, presented in Academy Navy envelope with gold wax seal
-

CHAPTER 9: THE BRAND VOICE & TONE

9.1 The Academy Voice

The Academy speaks with “**Scholarly Warmth**”—the precision of a medical journal combined with the compassion of a caring practitioner.

Characteristics:

- **Authoritative but Humble:** We state facts with confidence but attribute knowledge to Allah and His Messenger (peace be upon him) where appropriate
- **Precise but Accessible:** Medical terminology is used correctly but always defined on first use
- **Sacred but Practical:** Spiritual benefits are acknowledged alongside clinical evidence
- **Global but Rooted:** We serve a global ummah while honoring the classical Arabic tradition

9.2 Tone Modulation by Context

Context	Tone	Example
Clinical Protocol	Precise, Imperative	“Sterilize the cupping vessel for 15 minutes at 121°C.”
Student Encouragement	Warm, Supportive	“Your dedication to this sunnah is a blessing to your community.”
Academic Assessment	Objective, Constructive	“Demonstration of point location requires 90% accuracy for certification.”
Community Outreach	Inviting, Inclusive	“Discover the healing wisdom passed down through generations.”
Regulatory Correspondence	Formal, Exact	“The Academy certifies that all instructors meet [standard] as of [date].”

9.3 Prohibited Language

- Never guarantee cure or specific outcomes
- Never disparage conventional medicine
- Never use sensationalist or fear-based marketing
- Never attribute proprietary techniques to the Academy if they derive from classical sources
- Never use gendered language that excludes non-male practitioners (use “practitioner,” “student,” “instructor”)

CHAPTER 10: THE IMPLEMENTATION & GOVERNANCE

10.1 The Design Council

The Academy Design Council consists of:

- Chief Design Officer (Chair)
- Academic Director
- Clinical Director
- Head of Digital
- Student Representative (rotating, 1-year term)

All deviations from this Bible require unanimous Council approval.

10.2 The Version Control Protocol

- This Bible is versioned by year and revision (e.g., v2026.1)
- Minor revisions (typo corrections, contact updates) increment the decimal
- Major revisions (palette changes, new document families, structural overhauls) increment the year
- All Academy materials must reference the Bible version they comply with

10.3 The Compliance Audit

- Annual design audit of all active materials
- Random sampling of instructor-created materials
- Student feedback on visual consistency (surveyed annually)

- Digital accessibility audit (WCAG 2.1 AA compliance verified quarterly)
-

APPENDIX A (PART I): THE QUICK REFERENCE CARD

A one-page summary for all team members:

Never:

- Use colors outside the approved palette
- Set body text smaller than 12pt/16px
- Place the Academy Seal on busy backgrounds
- Use stock photography without approval
- Alter the proportions of the mihrab frame

Always:

- Maintain the breath margin (15% negative space)
 - Include the authority block
 - Use Sunnah Serif for headlines, Sunnah Sans for body
 - Check contrast ratios before publishing
 - Ask the Design Council if uncertain
-

APPENDIX B (PART I): THE TEMPLATE INDEX

Complete list of all master templates with file locations, version numbers, and responsible departments.
[To be maintained digitally and updated with each revision.]

APPENDIX C (PART I): THE SUPPLIER DIRECTORY

Approved printers, paper merchants, digital asset platforms, and type foundries. All suppliers must sign a compliance agreement acknowledging adherence to Academy standards.

PART II

THE SUNNAH REMEDIES ACADEMY EDUCATIONAL STANDARDS MANUAL

The Academic & Pedagogical Constitution

PREAMBLE

This Educational Standards Manual establishes the intellectual, pedagogical, and ethical framework within which all Sunnah Remedies Academy learning experiences are conceived, delivered, and evaluated. It is not merely a set of rules; it is a philosophy of education rooted in the Islamic tradition of seeking knowledge (ilm) with sincerity (ikhlas), excellence (ihسان), and beneficial application (amal salih).

Every course—whether Hijama, Prophetic Medicine, herbal studies, nutrition, or instructor certification—must align with the principles herein. This ensures that a student who completes any Academy program possesses not only technical competence but also the spiritual grounding, ethical sensitivity, and critical thinking capacity that distinguish a Sunnah Remedies practitioner.

CHAPTER 1: THE EDUCATIONAL PHILOSOPHY

1.1 The Foundational Axiom

“The best of you are those who learn the Qur’an and teach it.” (Sahih al-Bukhari)

The Academy operates on the conviction that education in the healing arts is an act of ‘ibadah when pursued with the intention of benefiting Allah’s creation. This transforms every lesson from mere information transfer into spiritual cultivation.

1.2 The Three Pillars of Academy Pedagogy

Pillar I: Sacred Grounding (‘Asl) Every course must root its content in the Qur’an and authentic Sunnah. This does not mean every lecture begins with a hadith as decoration; it means the *structure* of the course itself reflects divine wisdom. For example:

- Hijama courses teach the anatomical *and* spiritual dimensions of the points
- Nutrition courses ground dietary recommendations in the Prophetic diet *and* contemporary nutritional science
- Herbal courses honor the classical materia medica while engaging modern pharmacology

Pillar II: Evidence & Reason (‘Ilm) The Academy rejects the false dichotomy between “traditional” and “scientific” knowledge. Every therapeutic claim must be supported by:

- Primary classical sources (where applicable)
- Contemporary peer-reviewed research
- Clinical observation and case study
- Transparent acknowledgment of uncertainty where evidence is limited

Pillar III: Beneficial Practice (‘Amal) Knowledge that does not translate into improved patient care or community benefit is incomplete. Every course must include:

- Supervised practical application
- Case-based learning
- Community service components (where appropriate)

- Reflection on the spiritual state of the practitioner

1.3 The Student as Whole Person

The Academy educates not merely the intellect but the *whole person*:

- **Intellect:** Critical thinking, evidence evaluation, clinical reasoning
 - **Spirit:** Connection to Allah, sincerity in practice, patience with patients
 - **Body:** Personal health, self-care, understanding one's own constitution
 - **Community:** Ethical responsibility, cultural sensitivity, professional collaboration
-

CHAPTER 2: CURRICULUM ARCHITECTURE

2.1 The Unified Curriculum Framework

All Academy courses follow a **five-tier progressive structure**:

Tier 1: Foundations ('Usul)

- Duration: 20-40 hours
- Content: History, theology of healing, basic anatomy, safety, ethics
- Assessment: Written examination (80% pass threshold)
- Prerequisite: None

Tier 2: Theory ('Nazariyya)

- Duration: 40-80 hours
- Content: In-depth study of the discipline's theoretical framework, classical texts, modern science
- Assessment: Written examination + oral defense (80% pass threshold)
- Prerequisite: Tier 1 completion

Tier 3: Technique ('Mihna)

- Duration: 40-100 hours
- Content: Hands-on skill development, supervised practice, technique refinement
- Assessment: Practical examination + skill checklist (90% pass threshold for clinical techniques)
- Prerequisite: Tier 2 completion

Tier 4: Clinical Application ('Tatbiq)

- Duration: 60-200 hours (varies by discipline)
- Content: Supervised patient contact, case management, documentation, clinical reasoning
- Assessment: Case portfolio review + preceptor evaluation (85% pass threshold)
- Prerequisite: Tier 3 completion

Tier 5: Mastery & Teaching ('Tadris)

- Duration: 40-80 hours

- Content: Advanced theory, research methods, teaching methodology, instructor certification
- Assessment: Teaching demonstration + research project + peer review (90% pass threshold)
- Prerequisite: Tier 4 completion + 1 year clinical practice

2.2 Credit Hour Definition

One Academy Credit Hour equals:

- **Didactic:** 60 minutes of live or recorded instruction + 120 minutes of structured independent study
- **Laboratory/Practical:** 60 minutes of supervised hands-on practice + 60 minutes of preparation and reflection
- **Clinical:** 60 minutes of supervised patient contact + 30 minutes of documentation and case review

2.3 Cross-Disciplinary Requirements

Every student, regardless of specialization, must complete:

- **Islamic Bioethics Module** (8 hours): Covers fiqh of treatment, consent, gender relations in clinical practice, end-of-life considerations
- **Evidence Evaluation Module** (6 hours): How to read a research paper, understand bias, evaluate claims
- **Professional Communication Module** (4 hours): Documentation, referral protocols, interprofessional collaboration
- **Self-Care & Practitioner Wellness Module** (4 hours): Preventing burnout, maintaining spiritual practice, physical health

CHAPTER 3: TEACHING METHODOLOGY

3.1 The Pedagogical Mix

Academy courses employ a **blended learning model** with the following mandatory components:

A. Live Instruction (Synchronous)

- Minimum 30% of total course hours
- Instructor-led sessions with real-time interaction
- Mandatory attendance; missed sessions must be made up within 14 days
- Maximum class size: 24 students (to ensure individual attention)

B. Structured Self-Study (Asynchronous)

- Maximum 40% of total course hours
- Pre-recorded lectures, readings, case studies
- Must include knowledge-check quizzes every 20 minutes of content
- Progress tracking with automated reminders

C. Practical Application (Experiential)

- Minimum 20% of total course hours (clinical courses: minimum 40%)
- Hands-on practice with peer or instructor observation
- Video submission requirements for distance students (where applicable)
- In-person intensive requirements for clinical certification

D. Community & Reflection (Collaborative)

- Minimum 10% of total course hours
- Peer discussion forums, study circles, group case analysis
- Reflective journaling (submitted but not graded for content—graded for completion and depth)
- Mentorship check-ins with assigned faculty advisor

3.2 The Socratic-Clinical Method

Instructors are trained to use the **Socratic-Clinical Method**:

- Rather than lecturing for extended periods, instructors pose clinical scenarios
- Students must reason through diagnosis, treatment selection, and ethical considerations
- Incorrect answers are treated as learning opportunities, not failures
- The instructor's role is facilitator, not oracle

Example Exchange:

Instructor: “A patient with uncontrolled hypertension requests Hijama. What is your first consideration?” **Student:** “The safety of the procedure given the blood pressure.” **Instructor:** “Correct. What specific BP threshold would cause you to postpone? And what is the fiqh consideration if the patient insists?” **Student:** “Above 180/110... and that we do not harm, so we must refuse even if they insist.” **Instructor:** “Exactly. Now, how do you document this refusal?”

3.3 The Case-Based Learning Protocol

Every course must include a minimum of **12 clinical cases** (or equivalent for non-clinical courses: 12 applied scenarios) that progress in complexity:

Case Set	Complexity	Focus
Cases 1-4	Straightforward	Single issue, clear protocol, no complicating factors
Cases 5-8	Moderate	Multiple considerations, require prioritization, some ambiguity
Cases 9-12	Complex	Rare presentations, ethical dilemmas, interprofessional coordination required

3.4 The Classical Text Integration

For courses with classical foundations (Hijama, Tibb Nabawi, Herbal Medicine):

- Primary classical texts must be studied in structured excerpts, not merely referenced
- Students must learn to navigate Arabic terminology (with transliteration and translation)
- Classical recommendations must be critically evaluated against contemporary evidence
- The *chain of transmission* (isnad) for key narrations must be acknowledged

CHAPTER 4: ASSESSMENT & EVALUATION

4.1 The Assessment Philosophy

Assessment at the Academy serves three purposes:

1. **Verification:** Confirming that the student has achieved the learning outcomes
2. **Formation:** Providing feedback that shapes ongoing development
3. **Protection:** Ensuring that only competent practitioners receive certification (patient safety)

4.2 Assessment Types & Weightings

Formative Assessment (Ongoing)

- Weekly quizzes (auto-graded, unlimited attempts, must achieve 80% to proceed)
- Peer review exercises
- Reflective journal entries
- Instructor observation checklists during practical sessions

Summative Assessment (Terminal)

- Written comprehensive examination (multiple choice, short answer, essay)
- Practical skills examination (OSCE-style stations for clinical courses)
- Case portfolio (minimum 10 fully documented cases for clinical certification)
- Oral defense/viva voce (30-60 minutes with two examiners)

Weighting by Tier:

Tier	Formative	Summative Written	Summative Practical	Portfolio/Project
1 (Foundations)	30%	50%	10%	10%
2 (Theory)	25%	45%	15%	15%
3 (Technique)	20%	30%	40%	10%
4 (Clinical)	15%	25%	30%	30%

Table 7 – continued

Tier	Formative	Summative Written	Summative Practical	Portfolio/Project
5 (Mastery)	10%	20%	25%	45%

4.3 Pass Thresholds & Remediation

Assessment Component	Pass Threshold	Remediation Opportunity
Weekly Quizzes	80%	Unlimited retakes; must pass to unlock next module
Written Examination	80%	One retake after 30 days; different exam form
Practical Examination	90% (clinical) / 80% (non-clinical)	One retake after 60 days; additional supervised practice required
Case Portfolio	85%	Revision and resubmission; maximum two attempts
Oral Defense	80%	One retake after 30 days

Academic Integrity: Any instance of plagiarism, cheating, or misrepresentation of case documentation results in immediate suspension pending review. Second offense: permanent expulsion with no refund.

4.4 The Grading Scale

The Academy uses a **mastery-based grading system**:

Grade	Percentage	Descriptor
Mumtaz (Excellence)	95-100%	Exceptional mastery; eligible for honors designation
Jayyid Jiddan (Very Good)	85-94%	Strong competence; meets all certification requirements
Jayyid (Good)	80-84%	Satisfactory; meets minimum certification requirements
Maqbul (Acceptable)	75-79%	Below certification threshold; remediation required
Dha' if (Weak)	Below 75%	Does not meet standards; must repeat tier

No Curving: Grades reflect absolute standards, not relative performance. If all students in a cohort achieve Mumtaz, all receive Mumtaz. If none meet the threshold, none pass.

CHAPTER 5: INSTRUCTOR STANDARDS & EXPECTATIONS

5.1 Instructor Qualification Tiers

Tier A: Guest Lecturer

- Expertise in specific topic area
- May deliver 1-3 sessions per course
- Must complete Academy orientation (4 hours)
- Supervised by Course Director

Tier B: Associate Instructor

- Tier 5 certification in relevant discipline OR equivalent external qualification vetted by Academic Board
- Minimum 2 years clinical practice
- May teach Tier 1-3 content under supervision
- Must complete Academy Pedagogy Certificate (20 hours)

Tier C: Lead Instructor

- Tier 5 certification + 3 years teaching experience
- May design and deliver full courses independently
- Responsible for assessment design and grading
- Must maintain continuing education: 20 hours/year
- Must publish or present one academic work every 2 years

Tier D: Master Instructor

- 10+ years in field + 5+ years Academy teaching
- Authority to mentor other instructors
- Sits on curriculum review committees
- May serve as external examiner
- Expected to contribute to Academy research initiatives

5.2 The Instructor Code of Conduct

Every instructor signs and adheres to the following:

1. **Ikhlāas (Sincerity):** I teach for the sake of Allah, not reputation or income
2. **Amaanah (Trustworthiness):** I accurately represent my qualifications and the evidence base
3. **Rahmah (Mercy):** I treat every student with patience and respect, regardless of their starting point
4. **‘Adl (Justice):** I assess all students by the same standard, without favoritism
5. **Haya’ (Modesty):** I maintain appropriate boundaries, especially in mixed-gender educational settings
6. **Ihsan (Excellence):** I prepare thoroughly for every session and respond to student feedback
7. **Hifz al-‘Ilm (Preservation of Knowledge):** I do not misrepresent classical sources or fabricate evidence

5.3 The Classroom Environment

- **Gender Policy:** Separate seating for male and female students in live sessions; online sessions must offer gender-specific breakout rooms for practical demonstrations
- **Recording Policy:** All sessions recorded for student review; recordings retained for 2 years
- **Accessibility:** All materials provided in accessible formats; accommodations made for students with disabilities
- **Language:** Primary instruction in English; Arabic terminology taught with transliteration; translations provided where cohorts require

5.4 Instructor Evaluation

Students evaluate instructors anonymously after each course on:

- Knowledge depth and accuracy
- Clarity of explanation
- Responsiveness to questions
- Spiritual demeanor and adab
- Practical usefulness of content

Instructors scoring below 4.0/5.0 in any category must undergo supervised improvement plan.

CHAPTER 6: LEARNING OUTCOMES & COMPETENCIES

6.1 The Graduate Competency Framework

Every Academy graduate, regardless of discipline, must demonstrate:

Domain 1: Knowledge (What the graduate knows)

- K1: Foundational Islamic principles related to health and healing
- K2: Anatomy, physiology, and pathology relevant to their discipline
- K3: Classical theoretical framework of their discipline
- K4: Contemporary evidence base for their discipline
- K5: Legal and regulatory framework governing their practice

Domain 2: Skills (What the graduate can do)

- S1: Perform core techniques safely and effectively
- S2: Conduct thorough patient assessments
- S3: Develop individualized treatment plans
- S4: Document care accurately and comprehensively
- S5: Recognize limits of competence and refer appropriately
- S6: Educate patients and communities about health promotion

Domain 3: Attitudes & Behaviors (How the graduate practices)

- A1: Practices with sincerity and God-consciousness (taqwa)
- A2: Maintains patient confidentiality and dignity
- A3: Pursues continuing education and self-improvement
- A4: Collaborates respectfully with other healthcare providers
- A5: Advocates for ethical practice in their community
- A6: Maintains personal health and spiritual practice

6.2 Discipline-Specific Outcomes

Each discipline maps its curriculum to the six domains above, adding discipline-specific competencies. For example, a Hijama graduate must additionally demonstrate:

- Precise location of all Sunnah and therapeutic cupping points
- Proper sterilization and infection control protocols
- Contraindication screening and risk assessment
- Integration of Hijama with conventional medical care

CHAPTER 7: ACADEMIC QUALITY ASSURANCE

7.1 The Quality Cycle

The Academy operates a **Plan-Do-Study-Act (PDSA)** quality cycle:

Plan: Annual curriculum review with input from instructors, students, graduates, and external advisors

Do: Implement curriculum with fidelity to this Manual **Study:** Collect data on student outcomes, completion rates, graduate employment, patient safety incidents **Act:** Revise curriculum, assessment, or delivery based on data

7.2 External Validation

- **Advisory Board:** Composed of Islamic scholars ('ulama), medical doctors, researchers, and traditional medicine experts who review curriculum annually
- **Peer Review:** New courses reviewed by at least two external subject matter experts before launch
- **Accreditation:** Pursuit of recognized accreditation bodies for complementary medicine education

7.3 Student Success Metrics

The Academy tracks and publishes:

- Course completion rates (target: >85%)
- Certification pass rates on first attempt (target: >75%)
- Graduate employment/practice establishment rate at 6 months (target: >70%)
- Graduate satisfaction (target: >4.2/5.0)
- Patient safety incidents attributed to graduate practice (target: 0)

7.4 The Appeals Process

Students may appeal:

- Assessment grades (within 14 days of result; reviewed by independent examiner)
 - Academic integrity findings (within 7 days; reviewed by Academic Integrity Committee)
 - Instructor conduct concerns (submitted confidentially to Student Affairs)
-

CHAPTER 8: CONTINUING EDUCATION & LIFELONG LEARNING

8.1 Graduate Requirements

Certified practitioners must complete:

- **20 hours continuing education per year** (10 hours didactic, 10 hours practical)
- **Annual skills verification** (submitted video or in-person assessment)
- **Case review submission** (minimum 5 cases per year for clinical practitioners)
- **Ethics refresher** (2 hours every 2 years)

8.2 The Academy Alumni Network

Graduates join a lifelong learning community with:

- Access to updated course materials
 - Annual conference and refresher workshops
 - Peer consultation forums
 - Advanced specialization tracks
 - Instructor development pathway
-

APPENDIX A (PART II): COURSE DESIGN TEMPLATE

Standardized template for all new course proposals, including:

- Learning outcome mapping to Graduate Competency Framework
 - Content outline with hour allocations
 - Assessment plan with weightings
 - Resource requirements
 - Instructor qualification requirements
 - Estimated student workload
-

APPENDIX B (PART II): ASSESSMENT BANK PROTOCOL

Guidelines for developing, securing, and rotating examination questions to maintain integrity.

APPENDIX C (PART II): STUDENT HANDBOOK EXCERPT

Key policies on attendance, plagiarism, grievances, and accommodations.

PART III

THE SUNNAH REMEDIES ACADEMY CLINICAL STANDARDS MANUAL

The Patient Safety & Clinical Governance Constitution

PREAMBLE

This Clinical Standards Manual establishes the non-negotiable requirements for every clinical interaction, procedure, and patient care decision made under the auspices of Sunnah Remedies Academy. It exists to protect patients, support practitioners, and uphold the sacred trust (amaanah) that exists between healer and healed.

The Academy recognizes that its graduates practice in diverse regulatory environments—some with formal licensure for complementary medicine, others in unregulated or minimally regulated jurisdictions. This Manual sets the **minimum standard** regardless of local regulatory requirements. Where local law exceeds these standards, local law prevails. Where this Manual exceeds local requirements, Academy certification demands compliance with this Manual.

Every practitioner who holds Academy certification makes a covenant with every patient they serve: to practice safely, honestly, within their competence, and with the intention of pleasing Allah.

CHAPTER 1: SCOPE OF PRACTICE

1.1 The Boundary Doctrine

The Academy graduate practices within a **defined scope** that is:

- **Evidence-informed:** Based on classical sources, contemporary research, and clinical observation
- **Competency-limited:** The practitioner performs only procedures and gives advice for which they have been trained and assessed
- **Legally compliant:** Adherent to the laws of the jurisdiction in which they practice
- **Ethically bounded:** Guided by Islamic bioethics and professional integrity

1.2 Discipline-Specific Scopes

Hijama (Wet & Dry Cupping)

- Permitted: Assessment for cupping suitability, point selection based on Sunnah and therapeutic indications, sterile cupping procedure, post-procedure care, lifestyle advice
- Prohibited: Cupping over infected areas, active cancer sites (without oncologist clearance), open wounds, areas of severe vascular compromise; making medical diagnoses outside training; prescribing pharmaceutical medications; claiming to cure specific diseases

Prophetic Medicine (Tibb Nabawi)

- Permitted: Advising on Prophetic foods and practices (dates, black seed, honey, etc.), lifestyle counseling based on classical frameworks, spiritual health guidance
- Prohibited: Prescribing pharmaceutical doses of herbal preparations; advising patients to discontinue conventional medication; diagnosing medical conditions; practicing medicine without license where required

Herbal Medicine

- Permitted: Recommending traditional herbal preparations within established safety parameters, preparing simple formulations, educating on botanical properties
- Prohibited: Prescribing herbs that interact dangerously with common medications without pharmacist/physician consultation; manufacturing unlicensed pharmaceutical products; making unsubstantiated curative claims

Nutritional Counseling

- Permitted: Dietary assessment, meal planning based on Prophetic nutrition and contemporary science, lifestyle coaching, supplement recommendations within scope
- Prohibited: Prescribing medical nutrition therapy for complex conditions (diabetes, renal disease, eating disorders) without appropriate additional credentials; diagnosing nutritional deficiencies without laboratory confirmation

Instructor/Training Role

- Permitted: Teaching Academy-approved curriculum, assessing students, mentoring practitioners
- Prohibited: Certifying students in techniques for which the instructor lacks Academy authorization; teaching outside approved scope; granting credentials that appear to be medical licenses

1.3 The Red Line: When to Refer

Every Academy practitioner must refer the patient to a qualified medical doctor when encountering:

- Signs of acute medical emergency (chest pain, difficulty breathing, severe bleeding, altered consciousness)
- Conditions beyond their training (undiagnosed masses, neurological symptoms, psychiatric crisis)
- Requests for services outside their scope (prescription medication, surgical intervention, diagnostic imaging)
- Lack of improvement after reasonable course of treatment (defined per discipline)

- Any situation where the practitioner feels uncertain about safety

Documentation: All referrals must be documented in the patient record, including the reason for referral, the practitioner to whom referred, and patient acknowledgment.

CHAPTER 2: INFECTION PREVENTION & CONTROL

2.1 The Sterility Standard

The Academy mandates **Standard Precautions** for all clinical interactions, meaning every patient is treated as potentially infectious.

2.2 Hand Hygiene

- **Before and after** every patient contact
- **Before** any aseptic procedure
- **After** exposure to body fluids
- **After** touching patient surroundings
- Method: WHO 6-step handwashing with soap and water (minimum 40 seconds) or alcohol-based hand rub (minimum 20 seconds)
- Nails: Kept short, clean, no artificial nails for practitioners performing invasive procedures

2.3 Personal Protective Equipment (PPE)

Procedure	Minimum PPE
Patient assessment (non-invasive)	Clean uniform, hand hygiene
Dry cupping	Gloves, apron, eye protection if splash risk
Wet cupping (Hijama)	Sterile gloves, sterile gown, face shield or mask + eye protection, hair covering
Herbal preparation	Gloves, apron, eye protection when handling powders/liquids
Any procedure with splash risk	Full PPE as above

2.4 Instrument Sterilization & Disposal

Reusable Instruments (e.g., cupping vessels, glass cups)

1. Pre-clean: Remove visible debris with enzymatic detergent
2. Disinfect: Intermediate-level disinfectant (e.g., 70% isopropyl alcohol) for 10 minutes
3. Sterilize: Autoclave at 121°C for 15 minutes at 15 psi OR dry heat sterilization at 160°C for 2 hours
4. Store: In sterile packaging, dated, used within 30 days
5. Log: Maintain sterilization log with date, time, temperature, pressure, operator initials

Single-Use Items

- Lancets, blades, gauze, cotton: Used once, discarded in sharps container or biohazard waste
- Never reuse, never repack, never “sterilize” and reuse

Waste Management

- Sharps: Rigid, puncture-proof container, labeled, disposed per local regulations
- Bloody materials: Red biohazard bag, sealed, disposed per local regulations
- General waste: Regular disposal, no mixing with clinical waste

2.5 Clinical Environment

- Treatment surface: Wiped with intermediate-level disinfectant between every patient
- Floors: Cleaned daily with appropriate disinfectant
- Air: Adequate ventilation; HEPA filtration recommended for Hijama clinics
- Laundry: Used linens in sealed bag, laundered at 60°C minimum
- No food or drink in clinical treatment areas

2.6 Bloodborne Pathogen Exposure Protocol

If a needlestick, splash, or other exposure occurs:

1. Immediate: Wash area with soap and water; flush mucous membranes with water
 2. Report: Within 1 hour to supervisor or designated officer
 3. Evaluate: Source patient testing (with consent) for HIV, Hepatitis B, Hepatitis C
 4. Prophylaxis: Follow local post-exposure prophylaxis guidelines
 5. Documentation: Complete incident report; maintain confidentiality
 6. Follow-up: Medical evaluation at 6 weeks, 3 months, 6 months
-

CHAPTER 3: INFORMED CONSENT

3.1 The Consent Doctrine

Informed consent is not a signature on a form. It is a **process of communication** in which the patient understands:

- What will be done
- Why it is recommended
- What the expected benefits are
- What the possible risks are
- What alternatives exist (including doing nothing)
- That they have the right to refuse or withdraw consent at any time

3.2 Elements of Academy Informed Consent

For All Procedures:

1. **Nature of the procedure:** Clear, jargon-free explanation
2. **Purpose/benefits:** What the practitioner hopes to achieve

3. **Risks:** All material risks, including common side effects and serious but rare complications
4. **Alternatives:** Other treatments available, including conventional medical options
5. **Practitioner qualifications:** Academy certification level, scope of practice
6. **Confidentiality:** How records are kept and shared
7. **Right to refuse:** Explicit statement that consent is voluntary
8. **Questions:** Opportunity for patient to ask and receive answers

For Hijama Specifically:

- Explanation of wet vs. dry cupping
- Expected sensations (tightness, warmth, possible bruising)
- Risk of infection, scarring, dizziness, anemia (with frequent sessions)
- Contraindications checked and discussed
- Aftercare instructions provided

For Minors:

- Consent must be obtained from parent/guardian
- Assent (agreement) should be obtained from the minor if developmentally appropriate
- A chaperone must be present for all procedures on minors

For Vulnerable Adults:

- Additional safeguards; consent capacity assessment if cognitive impairment suspected
- Advocate or family member involved in decision-making
- Clear documentation of capacity assessment

3.3 Documentation of Consent

- **Written consent:** Required for all invasive procedures (Hijama, any skin-breaking technique)
- **Verbal consent:** Acceptable for non-invasive consultations; documented in notes with statement: “Verbal consent obtained after full explanation”
- **Consent forms:** Must be in language patient understands; interpreter used if needed
- **Retention:** Consent forms kept for minimum 7 years (adults) or until age 25 (minors)

3.4 The Academy Consent Form Template

All Academy practitioners must use the standardized consent form, which includes:

- Patient identification and contact information
- Procedure description
- Risk acknowledgment checklist
- Contraindication screening
- Practitioner certification number
- Date, signature of patient, signature of practitioner

- Witness signature (for invasive procedures)
-

CHAPTER 4: CLINICAL DOCUMENTATION

4.1 The Documentation Standard

“Write, for by Allah, if Allah punishes you, He punishes you for what your hands have written.”
(Adapted from Sahih al-Bukhari)

Clinical records are legal documents, quality improvement tools, and protections for both patient and practitioner. Every entry must be:

- **Contemporaneous:** Written at the time of the encounter or within 24 hours
- **Accurate:** Factual, objective, free from opinion stated as fact
- **Complete:** All required elements present
- **Legible:** If handwritten; typed preferred
- **Signed:** With practitioner name, certification number, and date

4.2 The Patient Record Structure

Initial Consultation Record:

1. Demographics: Name, DOB, contact, emergency contact, GP/primary physician
2. Presenting concern: Patient’s own words
3. History: Relevant medical, surgical, medication, allergy, family history
4. Assessment: Practitioner’s clinical findings (objective)
5. Contraindication screen: Checklist completed
6. Treatment plan: Proposed interventions, rationale, expected outcomes
7. Consent: Documentation of informed consent process
8. Education: Information provided to patient
9. Practitioner signature and date

Treatment Session Record:

1. Date and time
2. Session number (if series)
3. Subjective: Patient report since last session
4. Objective: Practitioner observations, vital signs if taken
5. Treatment provided: Specific techniques, points, preparations used
6. Patient response: Tolerance, immediate reactions
7. Adverse events: Any unexpected occurrences
8. Plan for next session

9. Home care recommendations
10. Practitioner signature and date

Discharge/Referral Summary:

1. Dates of treatment
2. Total sessions
3. Presenting concern and response to treatment
4. Final assessment
5. Referrals made (if any)
6. Recommendations for ongoing care
7. Follow-up plan

4.3 Record Retention & Security

- **Retention:** Minimum 7 years for adults; until age 25 for minors
 - **Storage:** Locked cabinet for paper; encrypted, password-protected, backed-up system for digital
 - **Access:** Only treating practitioner and authorized supervisors; patient has right to access their own records
 - **Disposal:** Shredded for paper; certified digital destruction for electronic records
 - **Transport:** Records never left in vehicles or taken to unsecured locations
-

CHAPTER 5: PATIENT SAFETY & RISK MANAGEMENT

5.1 The Safety Culture

The Academy cultivates a **just culture** in which:

- Errors and near-misses are reported without fear of blame (unless reckless or intentional misconduct)
- Systems are analyzed for root causes, not just individuals criticized
- Learning from incidents is shared across the Academy community
- Patient safety is prioritized over practitioner convenience or reputation

5.2 Contraindication Screening

Absolute Contraindications for Hijama:

- Hemophilia or other bleeding disorders
- Severe anemia (Hb < 8 g/dL) without medical supervision
- Active infection at treatment site
- Severe dehydration or shock
- Pregnancy (certain points; abdominal/lumbar cupping prohibited)
- Uncontrolled hypertension (>180/110 mmHg)
- Use of anticoagulant medication (relative; requires physician clearance)

Relative Contraindications (proceed with caution, physician consultation advised):

- Diabetes (risk of poor wound healing)
- Epilepsy
- Immunocompromised state
- History of keloid scarring
- Active skin conditions (eczema, psoriasis at site)

Mandatory Screening: Every patient must complete a standardized health questionnaire before first treatment. Practitioner must review and discuss any flagged items.

5.3 Adverse Event Management

Minor Adverse Events (expected, self-limiting):

- Bruising, mild discomfort, lightheadedness
- Management: Reassurance, aftercare advice, observation
- Documentation: Note in patient record

Moderate Adverse Events (requires intervention):

- Significant bleeding, prolonged dizziness, localized infection
- Management: Immediate first aid, medical referral if needed, follow-up within 24 hours
- Documentation: Detailed incident report, supervisor notification

Serious Adverse Events (life-threatening or requiring emergency care):

- Severe bleeding, anaphylaxis, syncope with injury, systemic infection
- Management: Emergency services activation, first aid, do not leave patient unattended
- Documentation: Full incident report within 24 hours, Academy Clinical Director notification within 2 hours, regulatory notification per local requirements

5.4 The Incident Reporting System

All adverse events, near-misses, and safety concerns must be reported via the Academy Incident Report Form within 24 hours. The form captures:

- Date, time, location
- Personnel involved
- Patient details (de-identified for aggregate analysis)
- Description of event
- Immediate actions taken
- Patient outcome
- Contributing factors
- Recommendations for prevention

Non-punitive Reporting: Self-reported errors are treated as learning opportunities. Failure to report a known incident is a serious breach of professional conduct.

CHAPTER 6: ETHICS & PROFESSIONAL CONDUCT

6.1 The Ethical Framework

Academy practitioners operate under **Islamic Bioethics**, which holds that:

- The body is a trust from Allah; it must be preserved and respected
- The patient has dignity ('izzah) that cannot be compromised
- The practitioner serves the patient, not the reverse
- Benefit (maslahah) must be maximized and harm (mafsadah) minimized
- Truthfulness (sidq) is obligatory in all professional dealings

6.2 Core Ethical Obligations

Beneficence (Ihsan)

- Act in the best interest of the patient
- Maintain competence through continuing education
- Practice within scope; do not experiment on patients
- Prioritize patient welfare over financial gain

Non-Maleficence (La Dharar)

- First, do no harm
- Recognize and disclose limitations
- Avoid treatments with risk exceeding likely benefit
- Report unsafe practices by others (duty to warn)

Autonomy (Ikhtiyar)

- Respect the patient's right to choose or refuse treatment
- Provide information necessary for informed decision-making
- Do not coerce, manipulate, or exploit vulnerable patients
- Respect cultural and religious preferences

Justice ('Adl)

- Treat all patients equally regardless of wealth, status, ethnicity, or gender
- Fair pricing; sliding scale for those in genuine need
- Do not withhold care based on ability to pay in emergency situations
- Advocate for equitable access to healthcare in the community

Confidentiality (Sitr)

- Patient information is sacred; never disclosed without consent
- Exceptions: Court order, imminent danger to self or others, mandatory reporting of abuse (per local law)
- Digital security: Encrypted storage, strong passwords, no sharing of login credentials
- Social media: Never discuss identifiable patients online, even with details changed

6.3 Boundary Management

Financial Boundaries:

- Fees transparent and agreed before treatment
- No hidden charges
- No kickbacks for referrals
- No selling of products under undue pressure

Social Boundaries:

- No romantic or sexual relationships with current or recent patients (minimum 2-year cooling-off period)
- No treating family members except in emergency (documented justification required)
- No accepting extravagant gifts (token gifts of nominal value acceptable with disclosure)
- No social media “friending” of patients on personal accounts

Religious Boundaries:

- May share general spiritual encouragement appropriate to Islamic context
- Never impose personal religious views or pressure patient toward specific practice
- Respect patient’s current spiritual state without judgment
- Distinguish between religious advice (refer to scholar) and health advice

6.4 Advertising & Public Communication

- All claims must be truthful and substantiated
- No before/after photographs without explicit, written consent
- No testimonials that guarantee outcomes
- No disparagement of conventional medicine or other practitioners
- Academy certification may be stated but must not imply medical licensure where none exists
- All promotional materials must carry disclaimer: “Sunnah Remedies Academy practitioners are certified in complementary therapies and do not replace licensed medical care.”

CHAPTER 7: CLINICAL GOVERNANCE

7.1 Governance Structure

Clinical Director: Overall accountability for clinical standards, incident investigation, quality improvement
Clinical Advisory Panel: Multi-disciplinary group (physician, scholar, experienced practitioners)

reviewing complex cases and policy **Peer Review Committee:** Practitioners reviewing each other's cases for quality and learning **Risk Management Officer:** Tracking incidents, analyzing trends, implementing preventive measures

7.2 Clinical Audit

- **Annual Case Review:** Every practitioner submits 10 cases for peer review
- **Standards Compliance Audit:** Unannounced review of clinic environment, documentation, consent processes
- **Outcome Tracking:** Patient-reported outcomes collected at baseline, mid-treatment, and follow-up
- **Benchmarking:** Comparison of outcomes across practitioners and sites to identify best practices and areas for improvement

7.3 Practitioner Performance Management

- **Competency Verification:** Annual skills assessment for invasive procedures
- **Continuing Education:** 20 hours/year minimum; log maintained and audited
- **Remediation:** Practitioners falling below standard enter supervised improvement plan
- **Suspension:** Immediate suspension for serious safety breaches, pending investigation
- **Decertification:** Permanent revocation of Academy certification for:
 - Gross negligence causing patient harm
 - Practicing outside scope resulting in harm
 - Fraudulent documentation or misrepresentation
 - Sexual misconduct
 - Repeated violations after remediation

7.4 Interprofessional Collaboration

Academy practitioners are expected to:

- Maintain respectful communication with conventional medical providers
- Share relevant clinical information (with patient consent) to support integrated care
- Accept referrals from and make referrals to medical doctors
- Participate in community health initiatives alongside other providers
- Never undermine patient confidence in their medical team

CHAPTER 8: EVIDENCE-INFORMED PRACTICE

8.1 The Evidence Hierarchy

The Academy evaluates therapeutic claims using the following hierarchy:

1. **Classical Islamic Evidence:** Authentic hadith with sound isnad; consensus of classical scholars; established practice in authoritative traditional texts
2. **Systematic Reviews & Meta-Analyses:** High-quality synthesis of multiple randomized controlled trials
3. **Randomized Controlled Trials (RCTs):** Well-designed experimental studies

4. **Cohort & Case-Control Studies:** Observational studies with appropriate controls
5. **Case Series & Expert Opinion:** Clinical observation and professional consensus
6. **Mechanistic/Preclinical:** Laboratory studies, animal models, theoretical rationale

Application: The strongest recommendations combine classical evidence with high-quality modern research. Where classical and modern evidence conflict, the Academy convenes its Advisory Board for deliberation.

8.2 The Evidence Evaluation Protocol

Before any new technique, product, or protocol is adopted into Academy curriculum:

1. **Literature Review:** Comprehensive search of classical and modern sources
2. **Critical Appraisal:** Assessment of study quality, bias risk, applicability
3. **Safety Review:** Evaluation of adverse event data
4. **Advisory Board Review:** Scholarly and clinical experts assess compatibility with Academy values
5. **Pilot Implementation:** Small-scale trial with close monitoring
6. **Full Integration:** Only after successful pilot and any necessary modifications

8.3 Research Conduct & Ethics

Academy-affiliated research must:

- Obtain appropriate ethical approval
- Follow Declaration of Helsinki principles
- Secure informed consent from research participants
- Register clinical trials before commencement
- Publish results regardless of outcome (no publication bias)
- Declare all conflicts of interest
- Protect participant confidentiality

8.4 The Limitations Doctrine

Academy practitioners must be honest about the limits of current knowledge:

- “The evidence for this practice is limited but promising”
- “This is a traditional practice with strong classical support but limited modern research”
- “This approach works for many patients but not all; individual response varies”
- Never claim “proven cure” for any condition
- Distinguish between “this may help” and “this will cure”

CHAPTER 9: SPECIAL POPULATIONS

9.1 Pediatric Patients

- Parent/guardian consent mandatory
- Practitioner must have specific pediatric training
- Modified techniques for smaller body size
- Chaperone always present
- Extra attention to emotional comfort and explanation

9.2 Elderly Patients

- Consider polypharmacy and drug-herb interactions
- Skin integrity assessment (increased fragility)
- Fall risk if lightheaded after procedure
- Cognitive assessment for consent capacity
- Slower pace, more frequent breaks

9.3 Pregnant & Lactating Patients

- Many herbs and interventions contraindicated
- Hijama absolutely contraindicated at certain points and stages
- Always consult with obstetric provider
- Extra caution with any substance that crosses placenta or enters breast milk

9.4 Patients with Chronic Disease

- Diabetes: Wound healing risk, neuropathy, infection susceptibility
- Cardiovascular disease: Blood pressure monitoring, anticoagulation status
- Immunocompromise: Infection risk, reduced response
- Mental health conditions: Trauma-informed care, appropriate boundaries

9.5 Cultural & Religious Sensitivity

- Understand patient's cultural background and health beliefs
- Accommodate gender preferences for practitioner (where possible)
- Respect fasting states and prayer times
- Provide halal-certified products when applicable
- Use interpreter services when language barriers exist

CHAPTER 10: EMERGENCY PREPAREDNESS

10.1 Emergency Equipment

Every clinical site must maintain:

- First aid kit (checked monthly, dated supplies)
- Automated External Defibrillator (AED) if local regulations require or recommend
- Emergency oxygen (if trained to administer)
- Emergency contact numbers prominently displayed

- Clear evacuation route

10.2 Emergency Protocols

All practitioners must be certified in:

- Basic Life Support (BLS) / CPR
- First Aid
- Anaphylaxis recognition and management (epinephrine auto-injector use)

Emergency Response Sequence:

1. Ensure scene safety
2. Assess patient responsiveness and breathing
3. Activate emergency services if indicated
4. Initiate appropriate first aid/CPR
5. Do not leave patient unattended
6. Notify Academy Clinical Director within 2 hours
7. Complete incident report within 24 hours

10.3 Emergency Medications

Practitioners may keep emergency medications (epinephrine, glucose) only if:

- Legally permitted in their jurisdiction
- They have training in administration
- Medications are in-date and stored properly
- Usage is documented and reported

APPENDIX A (PART III): CLINICAL FORMS LIBRARY

Complete set of standardized forms:

- Health History Questionnaire
- Contraindication Screening Checklist
- Informed Consent Form (general)
- Informed Consent Form (Hijama-specific)
- Treatment Record Form
- Incident Report Form
- Referral Form
- Discharge Summary Form
- Practitioner Self-Assessment Form

APPENDIX B (PART III): STERILIZATION LOG TEMPLATE

Standardized log for tracking sterilization cycles with required fields and signature blocks.

APPENDIX C (PART III): QUICK REFERENCE — EMERGENCY RESPONSE

Laminated card for clinical wall display with step-by-step emergency protocols and emergency contact numbers.

APPENDIX D (PART III): EVIDENCE SUMMARY TEMPLATES

Standardized formats for documenting literature reviews and evidence appraisals for new techniques or products.

PART IV

INTERCONSTITUTIONAL HARMONIZATION

These three Constitutions are designed as an integrated system:

Design Bible	Educational Standards	Clinical Standards
Dictates <i>how</i> Academy materials look	Dictates <i>how</i> Academy students learn	Dictates <i>how</i> Academy graduates practice
Ensures visual coherence across all documents	Ensures pedagogical coherence across all courses	Ensures safety coherence across all patient interactions
Governs templates, colors, typography	Governs curriculum, assessment, instructor conduct	Governs scope, safety, ethics, documentation
Protects institutional identity	Protects academic integrity	Protects patient welfare
Enforced by Design Council	Enforced by Academic Board	Enforced by Clinical Director

No course may be published without compliance verification against all three Constitutions.

No instructor may teach without certification in the Educational Standards.

No graduate may practice without certification in the Clinical Standards.

No material may be produced without certification in the Design Bible.

These three documents, taken together, form the **Magna Carta of Sunnah Remedies Academy**—the foundation upon which every future endeavor rests, and the standard by which the Academy’s contribution to the ummah will be measured.

MASTER APPENDIX: CROSS-REFERENCE INDEX

Design Bible → Educational Standards

- Document templates (Family B: Clinical Documentation) must align with Clinical Standards documentation requirements
- Video production standards must include informed consent disclaimers per Clinical Standards Chapter 3
- Color usage in assessment materials must meet accessibility standards per Educational Standards Chapter 4

Educational Standards → Clinical Standards

- Tier 3 practical examinations must include infection control assessment per Clinical Standards Chapter 2
- Case portfolios must demonstrate informed consent documentation per Clinical Standards Chapter 3
- Instructor qualifications must include clinical competency verification per Clinical Standards Chapter 7

Clinical Standards → Design Bible

- All clinical forms must use Family B template specifications
- Consent forms must include Academy Seal and authority block
- Emergency quick-reference cards must follow iconographic vocabulary standards

Clinical Standards → Educational Standards

- Scope of practice definitions inform curriculum boundaries
 - Incident reporting data feeds curriculum review cycle
 - Continuing education requirements align with graduate competency framework
-

RATIFICATION & AMENDMENT PROTOCOL

Ratification

These Constitutions become binding upon:

1. Approval by the Academy Executive Council
2. Endorsement by the Shari’ah Advisory Board
3. Publication to all Academy personnel and students
4. Integration into all Academy contracts and agreements

Amendment

- **Minor amendments** (typographical, contact updates, clarifications): Approved by relevant Council (Design, Academic, or Clinical)
- **Major amendments** (structural changes, new standards, policy shifts): Require unanimous Executive Council approval + 30-day comment period for stakeholders
- **Emergency amendments** (patient safety, legal compliance): Immediate effect by Clinical Director with retrospective Council ratification within 14 days

Version Control

- Master document version: Year.Major.Minor (e.g., 2026.1.0)
- All constituent parts share the same master version number
- Change log maintained in digital repository with attribution

“And say: My Lord, increase me in knowledge.” — Surah Ta-Ha, 20:114

THE THREE FOUNDATIONAL CONSTITUTIONS OF SUNNAH REMEDIES ACADEMY

Healing by the Book and the Sunnah

www.sunnahremedies.academy
